

Neurology Follow-up Report

Patient	Ian Woodward
Date of birth	1987-01-17
Referring	Dr. Amara Osei
Neurologist	Dr. Karen Walsh
Reason	Annual migraine review
Plan	Continue prevention

History

The patient is a thirty-nine year old man with chronic migraine diagnosed in 2019. Before preventive treatment he experienced three to four headache days per month. On the current regimen frequency has fallen to roughly one per month. Attacks are most likely on Monday mornings and on evenings when he teaches, consistent with stress and schedule load. Triggers include poor sleep, skipped meals, and fluorescent lighting.

Current treatment

Medication	Dose	Role
Topiramate	50 mg daily	Prevention
Sumatriptan	50 mg as needed	Acute relief

Examination

System	Finding
General	Well, no acute distress
Cranial nerves	Intact
Motor and sensory	Normal and symmetric
Coordination	Normal
Fundoscopy	No papilledema

Assessment

Chronic migraine, well controlled on current prevention. No red flags on history or examination. The reduction from three to four headache days per month to about one is a good response. Acute medication use remains low, which is reassuring.

Plan

- Continue topiramate at the current dose.
- Continue acute treatment at onset; limit to avoid overuse.
- Maintain consistent meals and sleep; the patient is coached on not skipping lunch.
- Keep a brief headache log noting day, trigger, and response.
- Return in one year or sooner if the pattern changes.

Lifestyle supports discussed

The patient maintains weekend walks in Sligo Creek Park and sings in his parish choir, both of which support sleep and stress management. He was encouraged to protect a regular lunch on busy work and teaching days, since meal skipping is a known trigger for him. Hydration and a steady caffeine routine were reviewed.

Coordination

This report is sent to the primary care physician for the shared record. Medication is reflected in the household tracker coded X08 and in the primary care summary coded P03. The next scheduled neurology follow-up is 2027-07-14.

Signed, Dr. Karen Walsh, Kingswood Neurology Associates.

Headache log guidance

A short log helps confirm that prevention is working and flags any change in pattern. The patient can keep a simple note for each headache using the columns below.

Field	What to record
Date and time	When the headache began
Likely trigger	Sleep, meals, stress, light
Severity	Mild, moderate, or severe
Treatment	Medication taken and time
Response	Relief and how long it took

When to call sooner

- A headache that is the worst ever or comes on suddenly.
- New weakness, numbness, vision change, or trouble speaking.
- A clear rise in frequency despite the preventive medication.
- Any concern about medication side effects.

This guidance supports the care plan and does not replace direct contact with the clinic for any urgent concern.